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VALUING CARE

Evidence from Chile's
"Caring for Those Who Care" Program

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Prepared By:

Angie Church

Elisabeth Schmeissner

Fifi Sangkhajorn

Nupur Savani



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I. Executive Summary

As Chile moves toward a national care system grounded in social and gender co-responsibility, community initiatives are essential for turning policy into real improvements for caregivers and families. Fundación Ronda's implementation of the "Caring for Those Who Care" pilot program addresses care gaps by pairing trained, paid care assistants with households dependent on unpaid family caregiving. The program aims to reduce familial caregiver overload while expanding formalized employment opportunities in the care sector, particularly for women.

This workshop project conducts a programmatic evaluation of the program's early implementation in Santiago, focusing on the experiences of paid care assistants, supervisors, and participating families. First, it examines the socio-economic impact of formalized care work on care assistants within Chile's broader care agenda, then zooms in on the realities of care assistants' mental well-being, workload, income opportunities, and daily routines. Third, it assesses program design factors such as operations, processes, and training, and finally, provides long and short-term recommendations for the implementing organization and policy recommendations for the government.

Using qualitative research methods, including interviews and focus groups with care assistants and supervisors, and an analytical framework based on the International Labour Organization (ILO)'s "5R Framework for Decent Care Work", the project provides practical recommendations and monitoring tools to strengthen implementation, support learning, and build evidence for future adaptation or expansion of an equitable and sustainable care system in Chile.

The team found that the program "Caring for Those Who Care" is highly successful at empowering women socially, emotionally, and financially to become paid care assistants within their communities, which strengthened their social bonds and emotional satisfaction and gave them dignified work opportunities. The team also found that the scope of the program was not enough to alleviate the needs of care assistants for adverse mental health, day-to-day challenges, intake operations (including training), or hidden financial costs.



To address these challenges, the team recommends 1) utilizing peer support groups; 2) workload monitoring and restructuring for mental health and work-life balance; 3) matching care assistants to families based on past experience and creating a comprehensive training program to better use care assistants' existing skills and providing opportunities for professional development; and 4) reducing hidden financial costs of work, among other recommendations.

Finally, recognizing the precarious nature of this program's future and the lives of care assistants, caregivers, and care recipients that depend on it for survival, the team recommends that the city government of Santiago and the national government of Chile dedicate more resources to continuing, improving, and expanding the program to include more households and more care assistants throughout the region and the country. Because unpaid care work is worth 25.6% of Chile's GDP, formalizing care through programs such as "Caring for Those Who Care" has the potential to expand the labour force and workforce participation rate for women, resulting in increased economic activity and growth worth one-fourth of the Chilean economy.

II. Client Agency

Fundación Ronda is a Chile-based nonprofit organization dedicated to inclusion and empowerment for people with disabilities and their caregivers. Headquartered in Santiago, the organization has also implemented programs in regions including Puerto Montt, Valparaíso, Los Lagos, and Coquimbo. Its initiatives focus on supporting independent living, socio-labor inclusion, and access to employment opportunities for people with disabilities.

In 2021, Fundación Ronda launched "Main Characters," a program designed to provide women caregivers with self-care and personal development tools to reduce overload, improve emotional well-being, and promote autonomy. To date, 310 caregivers from 12 municipalities have participated.

Fundación Ronda's programs are supported through partnerships with the government of Chile, including "Chile Cuida" (Chile Cares) and the Ministry of Social and Family Development, as well as private donations. This project focuses on its "Caring for Those Who Care" program, implemented with the Santiago Regional Government.



III. Background

International Context

Worldwide, care work remains heavily unpaid and unequally distributed, with women performing around 76% of unpaid care work (Gomes, 2024). Across Latin America, many women engaged in unpaid caregiving are unemployed, underemployed, working informally, or living in low-income households, increasing vulnerability to poverty, debt, and weak social protection systems (Serafini et al., 2024). Governments are increasingly recognizing the care economy as a public good, a social right, and an emerging productive sector that requires greater public investment and policy coordination (Serafini et al., 2024).

Global and regional initiatives reflect this shift. In Latin America, the Tlatelolco Commitment calls for comprehensive care systems, improved labor protections, and recognition of unpaid and informal care work as a driver of inequality (Tlatelolco Commitment, 2025). Several countries, including Uruguay, Mexico, Colombia, Brazil, and Panama, are expanding care infrastructure, training, and public services to reduce burdens on women caregivers (UN Women, 2025; Papworth, 2025). These regional efforts are highlighted in the following table.

Table 1: Regional Care Economy Initiatives

<u>Country & Program</u>	<u>Key Features</u>	<u>Why It Matters</u>	<u>Relevance for Chile</u>	<u>Sources / More Info</u>
Uruguay: National Integrated Care System (SNIC)	Comprehensive, state-led system covering childcare, elder care, and disability services; formal training and certification for care workers	Considered a regional leader in institutionalizing care as a public system; reduces the unpaid care burden and supports female labor participation	Demonstrates the value of a coordinated national care system with standardized roles and training—Chile could strengthen alignment between programs like Chile Cuida and community	SNIC Uruguay



			initiatives such as Fundación Ronda through clearer national frameworks	
Mexico: National Care System (in development)	Cross-ministerial coordination; recognition of care as a social right; expansion of services for dependents and caregivers	Reflects a shift toward treating care as a rights-based policy area rather than informal support	Highlights the importance of legal recognition and policy framing—Chile could further formalize care as a right, strengthening political commitment and long-term funding stability	National Care System Mexico
Colombia: Manzanas del Cuidado (Care Blocks)	Community-based service hubs offering training, respite care, and services for both caregivers and care recipients	Directly addresses caregiver time poverty and well-being through localized service delivery	Offers a model for scaling community-based care centers—Chile could expand similar neighborhood-level support structures to reduce fragmentation and improve access for caregivers in different municipalities	Manzanas del Cuidado de Bogotá
Brazil: Bolsa Família + Childcare Expansion	Conditional cash transfers combined with increasing investment in early childhood care services	Integrates social protection with caregiving needs, reducing poverty while supporting families	Suggests the importance of linking financial support with care services—Chile could better integrate income support with caregiving	Bolsa Familia Brasil



			programs to reduce out-of-pocket costs for families and care assistants	
Panama: National Care Policy	Early-stage policy focused on expanding services and professionalizing care work	Reflects growing regional momentum toward formal care systems	Reinforces that even early-stage systems prioritize workforce formalization and service expansion, both of which are key areas for strengthening Chile's current care infrastructure	National Care Policy Panama

National Context

In Chile, unpaid caregiving remains a major source of gender inequality. Women perform most care work, spending an estimated 22.1 to 42.8 hours weekly on unpaid caregiving, compared with 6.7 to 19.8 hours for men (Milesi, 2024). This limits women's access to paid employment, education, rest, and financial independence. Many caregivers leave the workforce or absorb high personal costs, despite unpaid care contributing an estimated 25.6% of Chile's GDP (UN Women, 2025). Women still perform 71.7% of all care work and face higher unemployment and informality (Baspineiro, 2024).

The scale of care demand is significant. Around 17.6% of Chilean adults (approximately 3.4 million people) live with a disability, increasing reliance on family caregivers (Milesi, 2024). During the COVID-19 pandemic, nearly 1.5 million women exited the labor market because they could not balance paid work with unpaid caregiving (Baspineiro, 2024). In response, the government launched Chile Cuida, with more than 85,000 caregivers enrolled, most of them women (Milesi, 2024). Chile is also considering a National Care and Support System that would recognize care as a guaranteed right, expand services, and professionalize care work.



Local Context

Santiago's changing demographic profile and social dynamics highlight the need for a more coherent and comprehensive care-economy policy. The metropolitan area has more than 6.6 million residents and a dependency ratio of 53%. The old-age dependency ratio stands at 23.6%, while youth dependency remains high at 29.46% (Urban and Cities Platform of Latin America and the Caribbean, 2025). These trends increase household care burdens, particularly where affordable formal services remain limited. As Chile's largest urban center, Santiago is a critical setting for testing community-based care models that connect households with formal support systems.

Sector-Specific Issues

Chile's care economy has historically followed a “familistic” model in which families (primarily women) absorb care responsibilities because of limited public services and low male participation in domestic work (Domínguez-Amorós et al., 2021). This has produced a deeply feminized system of unpaid care and lower female participation in paid employment. Even when households hire help, women often retain the mental load of organizing and supervising care (Domínguez-Amorós et al., 2021).

Current policy efforts, including *Chile Cuida* and the proposed National Care and Support System, aim to shift toward a model based on social and gender co-responsibility by integrating the state, market, family, and community (Ministerio de la Mujer y la Equidad de Género, 2024). Within this transition, programs such as the Regional Government of Santiago's “Caring for Those Who Care”/”[Cuidando a Quienes Cuidan](#)” program help bridge policy goals and lived realities by reducing caregiver overload, improving well-being, and supporting more equitable care arrangements.

IV. Rationale

The “Caring for Those Who Care” program covers 20 municipalities within the Santiago Metropolitan Region. Fundación Ronda is one of many implementers under this program that matches trained, paid care assistants with families that have high



care needs. This creates employment opportunities for people who are not in the labor market, while meeting the needs of families. This SIPA workshop conducted a formative assessment of the program's early implementation by examining the experiences of care assistants and their supervisors. The team assessed initial impacts on well-being, economic opportunities, and daily routines to understand how the program functions in practice and what changes it produces for caregivers and households.

This project identified opportunities to strengthen implementation by exploring operational challenges, professional needs, and institutional barriers through structured interviews with care assistants, supervisors, and policy experts. These insights from focus groups and interviews inform the practical, evidence-based recommendations listed in the report that seek to improve policy and program design. Overall, the project aims to support the “Caring for Those Who Care” in refining its care model and generating evidence for potential improvement and future scaling.

V. Objectives

The SIPA workshop team conducted a formative assessment of the “Caring for Those Who Care” program implemented by Fundación Ronda. The assessment focused on participant experiences, program effectiveness, and operational gaps to generate actionable recommendations to strengthen implementation and inform future monitoring and scaling. By examining how program activities translate into improved caregiver well-being and program performance, the study supports Fundación Ronda's efforts to enhance impact within the broader care economy framework.

Targeted Objectives

1. Assess paid care assistants' experiences and program effectiveness by examining their engagement, workload, well-being, job security, professional development needs, and the program's effects on employability, skills, and economic opportunities.
2. Assess operational, structural, and institutional factors that shape program delivery and influence overall effectiveness.



3. Develop evidence-based recommendations to strengthen program design, delivery, and participant support, and identify policy adjustments to better align with broader care economy objectives.

Theory of Change

If the program adopts targeted psycho-social and capacity-building support for caregivers and streamlines operations and program design, then this will lead, in the short term, to improved well-being, reduced emotional and time-related overload, and greater autonomy, and in the long term, to strengthened care assistant engagement, improved program delivery and operational effectiveness, better systematic monitoring and learning, and greater program sustainability, policy alignment, and potential scalability within Chile's evolving care economy framework. This is because stronger support systems for care assistants and efficient operations reduce burdens on both care assistants and supervisors, improving retention rates and service delivery, and strengthening the program's long-term scalability and sustainability.

VI. Methodology

This study uses a qualitative research design to examine how the “Caring for Those Who Care” program is experienced by paid care assistants and supervisors, and how these experiences shape program effectiveness. The approach focuses on participants' lived experiences of care work, well-being, and employment conditions, while also exploring how program design and delivery influence outcomes.

The research is structured around four thematic pillars: (1) **Socio-Economic Impact**, including recognition, empowerment, and employability; (2) **People**, including mental health and work-life balance; (3) **Program**, including design and training; and (4) **Operations**, including financing, contracts, and standard operating procedures.

Methodologically, the study combines desk research, semi-structured interviews, and small focus groups conducted during a one-week field visit in Santiago. Approximately 50 people participated: supervisors, care assistants, thematic experts, and policy stakeholders. Data was analyzed thematically to identify patterns across groups and inform actionable recommendations. The interview and focus group guides were



submitted to the IRB to satisfy ethical considerations, which also included voluntary participation, confidentiality, and minimizing power imbalances.

Quantitative tools were not used for primary data collection.

VII. Research Findings and Recommendations

The research findings and recommendations are structured along four key axes of analysis, capturing both the human and systemic dimensions of the program. The first axis, socio-economic impact, explores outcomes related to recognition, empowerment, and employability. The second axis, people, centers on participant experiences, with particular attention to mental health and work–life balance. The third axis, program, assesses the effectiveness of design and training in delivering skills and support. The fourth axis, operations, examines the systems underpinning implementation, including financing, contractual arrangements, and standard operating procedures. Together, these axes of analysis provide a comprehensive framework for evaluating both program outcomes and areas for improvement.

1. Socio-Economic Impact

1.1 Social Gaps: Research Findings

Care assistants in the “Caring for Those Who Care” program fill gaps in public services for care recipients and provide physical and psychosocial support to their caregivers.

There are numerous social and policy gaps surrounding care work and support for individuals with disabilities. Without programs like “Caring for Those Who Care”, there is a failure to provide adequate long-term assistance to either care recipients or familial caregivers, leaving many families to navigate complex care requirements without support, financially or otherwise.

According to key informants like Carlos Kaiser, a disability rights activist, individuals with disabilities are frequently abandoned by both the state and their broader communities, forcing them to rely on themselves or immediate support networks for care despite significant needs that may not be satisfied by that level of support. In



this context, access to care can be the key factor in maintaining survival and quality of life.

Several care assistants who work with disabled individuals and their families through Fundación Ronda in the “Caring for Those Who Care” program describe their work as a critical lifeline wherein they are the only people preventing some care recipients, whose caregivers also suffer from disability or have passed away, from being completely abandoned.

At the same time, the burden of caregiving typically falls on women who often lack financial compensation and physical and emotional support. They must care for family members without formal institutions to help them, and as a result, may suffer from stress and economic insecurity.

Programs such as “Caring for Those Who Care”, therefore, are necessary in filling these systemic social gaps through which both caregivers and care recipients can fall. Care assistants act as a lifeline for caregivers and care recipient dyads by providing physical and psycho-social support, taking some of the burden off of the caregivers, and thereby ensuring that the care recipients get the level of care and attention they need.

1.2 Social Gaps: Recommendations

Short-Term Recommendations

Based on the International Labor Organization's 5R framework for decent care work, the following recommendations should be taken into consideration to strengthen the program's socio-economic impact:

1. Formally recognize the role of women in care work

Actively celebrating the work and stories of care assistants can help reinforce their value within the program. This could include storytelling campaigns highlighting care assistants' contributions through social media posts, newsletters, or internal communications, as well as organizing small recognition moments during team meetings or larger events such as year-end gatherings. Sharing these stories both acknowledges the effort and impact of individual care assistants and contributes to a



larger discussion about the positive impact of paid caregiving for care assistants and families receiving care.

2. Reduce and redistribute unpaid care work from female family members to social services, and pursue gender equality in care work

The work of this program is successful in its initial goal of improving care and support through public programs and should be continued and expanded, in order to continue reducing and redistributing workload away from the individual and towards the community. At the same time, recruitment efforts should market job advertisements to men as well as women, and emphasize the gender-neutral necessity of care work. This will destigmatize care as “women’s work” and emphasize that care work is a universally necessary social responsibility that can fall on both men and women.

1.3 Empowerment & Employability: Research Findings

Care assistants find their work meaningful and empowering, but many lack the ability to circumnavigate broader gaps in the care system that create difficult working conditions for them.

Care assistants find their work deeply personally rewarding and socially and economically empowering. They report that their work gives them a sense of purpose in restoring the dignity and livelihoods of caregivers and their families, and they find dignity in being paid as caretakers. Care assistants tend to be highly motivated to participate in “Caring for Those Who Care” and work to the best of their ability. However, motivation can mask overwork. A strong sense of purpose leads care assistants to accept difficult conditions, burn out, or break rules, because they feel personal responsibility for the well-being of families and care recipients, sometimes beyond what the program is able to provide for them. For example, some assistants reported texting their caregiver counterparts while they were off the clock to help them navigate bureaucracies like school registration, though they were not supposed to. Because they feel committed to the families they care for, they have a hard time “switching work off,” which can negatively impact working conditions.

Care assistants highlight that their personal histories of care influence their professional empathy, thereby strengthening the bond with the dyads being cared for.



Many have experience taking care of their own family members battling illnesses like cancer and dementia, so they are aware of the burdens the caregivers and care recipients face and are equipped to help. Personal experiences, however, can also reactivate past traumas, creating an emotional burden that exists alongside the satisfaction derived from the work.

In the broader scope of carework, care assistants go above and beyond to fill social-service gaps, particularly for marginalized communities like refugees and immigrants, low-income families, and disabled individuals, many of whom remain underserved by national welfare systems and lack access to essential support services. This program empowers the care assistants to care for communities that they feel strongly connected to, but it is a temporary stopgap for necessary public services. The risk of reliance on care assistants' empathy and willingness to work extra is that it can obscure the state's responsibility to provide financial or other forms of care, and normalize underinvestment in public care systems.

1.4 Empowerment & Employability: Recommendations

According to the ILO 5R framework, the program should reward and represent paid care assistants by strengthening their sense of empowerment and employability through the following recommendations:

Long-term Recommendation

1. Establish Promotion Pathways and professional development opportunities

Establishing defined promotion pathways and professional development opportunities would create a stronger sense of long-term opportunity within the program. This could involve partnering with local organizations or community colleges to offer accessible, paid training programs and certifications in areas such as caregiving, social services, or health support. Linking these opportunities to career progression, such as senior care assistant roles, supervisory positions, or specialized certifications, would provide tangible incentives for skill development. However, ensuring that training is compensated and scheduled in a way that accommodates care assistants' existing responsibilities will be key to participation.



2. People: Mental Health and Work-Life Balance

2.1 Mental Health: Research Findings

Care work provides strong meaning and connection while also generating sustained emotional strain for both care assistants and supervisors.

Meaning, Recognition, and Emotional Connection

Most of the paid care assistants shared that this initiative has given their lives meaning and purpose. Many said that this job requires empathy and the desire to help people. They are motivated in their role as care assistants because of their desire to help people, and they find the work fulfilling.

Many emphasized the satisfaction of providing comfort and companionship to individuals in vulnerable situations, as well as the sense of purpose derived from supporting families during periods of need. The relationships formed with care recipients were frequently described as trusting and reciprocal, with workers feeling appreciated and emotionally connected within the households they support.

This sense of recognition and human connection emerged as a key source of motivation and job satisfaction. Participants often framed their work as extending beyond routine tasks, emphasizing the emotional and relational dimensions of care. For many, these connections reinforced the perceived importance of their role and helped sustain engagement despite broader challenges within the work environment. Supervisors similarly described finding meaning in supporting care assistants and helping maintain continuity of care across households, reinforcing the relational nature of the program at multiple levels.

Emotional Strain and Limited Institutional Support

At the same time, emotional strain remains a persistent dimension of the job. Care assistants often act as “emotional containers” within households, absorbing stress, anxiety, and emotional distress from both care recipients and family members while managing their own emotional responses. This dual responsibility creates ongoing emotional pressure that accumulates over time, even in otherwise supportive environments.



Participants also highlighted the emotional difficulty of dealing with the death of care recipients, particularly when long-term relationships had been formed. These experiences were described as deeply personal, with emotional impacts extending beyond the workplace.

Supervisors reported parallel forms of strain. In addition to administrative responsibilities, they described feeling emotionally overwhelmed by heavy workloads, constant crisis management, and the pressure of supporting both care assistants and families. Many noted limited opportunities for rest or recovery, alongside a lack of structured institutional mental health support. High staff turnover further intensified their burden, while the cumulative emotional demands of supervision, such as managing conflict, responding to crises, and witnessing repeated loss, remain largely invisible within organizations.

2.2 Mental Health: Recommendations

2.2.1 Short-Term Recommendations

Strengthen mental health support and manage emotional workload through structured services, peer support, and systematic workload monitoring.

In the short term, improving mental health outcomes should focus on providing accessible support services while integrating emotional workload into day-to-day program management for both care assistants and supervisors.

1. Provide access to confidential mental health services

Priority should be given to expanding access to mental health support through partnerships with licensed psychologists. Each care assistant and supervisor should be offered a minimum of three free counseling sessions annually, with services designed to be confidential, easily accessible, and flexible enough to accommodate varying work schedules. Establishing a low-barrier entry point for psychological support can help normalize help-seeking behaviors, reduce stigma surrounding emotional distress in care work, and strengthen the long-term well-being and resilience of care personnel operating in emotionally demanding environments.

2. Institutionalize structured peer-support groups



The program should establish regular peer-support groups (e.g., monthly or biweekly) facilitated by trained professionals. These groups should provide space for emotional processing as well as collective reflection on shared challenges, including grief, burnout, and boundary management. In care settings where emotional strain is often internalized, structured peer spaces can reduce isolation and strengthen informal support networks among both assistants and supervisors.

3. Implement formal debriefing protocols for high-stress events

Facilitated debriefing sessions should be mandatory following particularly traumatic events, such as the death of a care recipient or other critical incidents. These sessions should take place within a defined timeframe and be led by trained facilitators. Formalizing debrief processes ensures that emotional strain is addressed at key moments and helps prevent the accumulation of unresolved stress over time.

4. Integrate workload monitoring and case review systems

The program should introduce a structured workload monitoring system that incorporates emotional intensity as a core factor in assignment decisions. This should include regular case reviews (e.g., monthly) where supervisors assess the emotional and physical demands of each placement using clear criteria, such as end-of-life care, severe disability support, or crisis situations.

Based on these assessments, high-intensity cases can be flagged, and supervisors can rotate assignments to prevent prolonged exposure without recovery periods. Incorporating a formal review process ensures that workload adjustments are consistent, transparent, and responsive to changing conditions across households. This approach recognizes emotional labor as an integral component of workload and embeds mental health considerations directly into program operations.

2.2.2 Long-Term Recommendations

Institutionalize mental health support and embed emotional workload into long-term workforce design and sustainability.



Over the longer term, improving mental health outcomes will require moving beyond ad hoc support and embedding psychosocial well-being into the program's structure, workforce policies, and long-term planning.

1. Institutionalize mental health support within program design

Mental health support should be embedded as a core component of human resource policy and program design. This includes integrating psychosocial support into annual budgeting processes, with dedicated funding for counseling services, supervisory capacity, and preventive well-being initiatives. Formalizing these supports would ensure that mental health is treated as an ongoing operational priority rather than a reactive intervention. Over time, this approach would help position emotional well-being as an integral dimension of care quality and program sustainability.

2. Develop workload flexibility and role rotation pathways

The program should introduce structured career pathways that recognize the long-term emotional demands of care work. This could include optional role rotation systems that allow care assistants and supervisors to periodically shift between higher- and lower-intensity cases. Such rotations would help distribute emotional burden more equitably, reduce burnout risk, and support sustained engagement in the sector. Establishing clear pathways also signals that long-term retention requires flexibility, not just endurance.

3. Introduce structured rest periods and sabbatical policies

To support long-term recovery, structured policies for planned rest periods or short-term sabbaticals should be introduced, particularly for staff with sustained exposure to high-intensity cases. These may include partially or fully paid leave options (e.g., one to four weeks), linked to tenure or cumulative workload exposure. Where full compensation is not feasible, hybrid arrangements such as partial pay or job-protected unpaid leave can still provide meaningful opportunities for recovery. Clearly defined eligibility criteria and scheduling procedures would help ensure equitable access to these breaks while safeguarding program continuity and operational stability.

4. Create alternative career pathways within the care sector



The program should also develop pathways for experienced staff to transition into less emotionally intensive roles over time, such as training, coordination, or administrative support. This approach enables the retention of institutional knowledge while reducing prolonged exposure to emotionally demanding frontline responsibilities. Offering such transition opportunities can enhance workforce stability and sustainability, while also recognizing that long-term engagement in care work may require progressive role adaptation rather than continuous frontline assignment.

Together, these long-term measures aim to shift the program from reactive emotional support toward a proactive model of workforce sustainability, where mental health is embedded in how work is structured, supported, and sustained over time.

2.3 Work-Life Balance: Research Findings

Work-life balance is a challenge for most care assistants and supervisors, driven by fragmented schedules, travel demands, and limited structure.

Fragmented Schedules and Travel Demands

Care assistants and supervisors both described challenges in maintaining a sustainable work-life balance, largely driven by how time is structured within the program. For assistants, the workday is often fragmented across multiple households in different locations, requiring frequent travel between placements. This extends the workday beyond direct care hours and creates ongoing time pressure. Many participants explained that scheduled breaks, including lunch, are often used just to commute, leaving little time to rest, eat, or recover during the day.

Daily schedules are also highly unpredictable. Travel times vary depending on distance, transportation availability, and delays, making it difficult for assistants to plan their day or consistently arrive on time. In some cases, assistants described waiting outside homes due to delayed entry or last-minute cancellations, which reduces usable time and adds inefficiency to already tight schedules. These disruptions can also create tension with families, especially when delays are outside of the assistant's control.



Limited Structure and Expanding Responsibilities

Supervisors face similar challenges in managing time across the program. In addition to coordinating schedules, they are responsible for responding to real-time issues, handling cancellations, and supporting assistants across multiple households. This often requires them to be available throughout the day and, in some cases, outside of standard working hours. The lack of clearly defined limits on availability or caseload contributes to fragmented and unpredictable workdays for supervisors as well.

At the same time, there is limited structure guiding how time and responsibilities are managed across both roles. Assistants are expected to adapt to each household without a consistent system for prioritizing tasks, while supervisors must balance coordination, oversight, and problem-solving without standardized workload guidelines. This creates uneven demands throughout the day and makes it difficult to maintain stable routines.

Implications for Work-Life Balance

These patterns suggest that work-life balance challenges are closely tied to how work is organized across the day. Long, segmented schedules, combined with travel demands and limited protected break time, make it difficult for both care assistants and supervisors to sustain consistent routines or recover between tasks.

2.4 Work Life Balance: Recommendations

2.4.1 Short-Term Recommendations

Strengthen time and workload management through clearer structure and scheduling.

These recommendations address the key drivers of work-life balance challenges identified above, particularly fragmented schedules, limited structure, and persistent time pressures.

1. Implement daily task checklists and define task boundaries within contracts

Introducing greater flexibility and agency within employment contracts can empower care assistants and maintain a stable work-life balance. Care assistants should be given the opportunity to specify in writing which additional daily tasks they feel



comfortable taking on and which fall outside their preferred scope of work, based on their previous experiences and current willingness. In practice, assistants often expand their roles in response to immediate needs in the home, which contributes to longer and less predictable workdays. In creating task lists for themselves, some may express their willingness to assist with childcare-related duties, while others may prefer not to take on tasks such as pet care or errands unrelated to caregiving, for example. Establishing clearer task boundaries would help contain workload within scheduled hours and prevent unplanned increases in workload and time demands, reducing misunderstandings, role strain, or the need for reactive coordination by supervisors.

2. Adjust scheduling practices to reflect time demands

Scheduling practices should be adjusted to better reflect how time is actually spent across the workday to protect working time from compression caused by travel and coordination demands. For assistants, this includes incorporating travel buffers between households so that commuting time does not reduce protected working time or extend total time demands across the day. For supervisors, schedules should explicitly allocate time for coordination, issue resolution, and communication across placements. Grouping households geographically and minimizing long-distance transitions within a single day can further reduce time fragmentation and support more stable routines. These changes would improve predictability and help both roles manage their time more effectively.

3. Establish and enforce daily rest

Daily rest periods should be formalized through clear policy and scheduling procedures. Assistants should have at least one protected break of a defined minimum length (e.g., 30–60 minutes), scheduled in advance and not used for travel time. Schedules should explicitly block this time, and supervisors should ensure that placements and routing arrangements allow breaks to be taken as intended. For supervisors, rest protections should include defined working hours and limits on responding to non-urgent issues outside of those hours, supported by clear escalation protocols for urgent situations.



Implementation should be supported through regular supervisory check-ins and simple monitoring procedures. Monthly meetings can be used to review schedules, identify missed or shortened breaks, and adjust assignments where time demands consistently exceed planned hours. Standardized guidelines for arrival windows, delay communication, and cancellation notice periods can further reduce disruptions that erode protected time. Together, these measures help ensure that rest time is consistently maintained and that both assistants and supervisors can manage their workload more sustainably.

2.4.2 Long-Term Recommendations

Redesign scheduling systems and workload standards to reduce fragmentation and support sustainable time management.

Over the longer term, improving work-life balance will require structural changes to how time and workload are allocated, tracked, and managed across both care assistants and supervisors.

1. Consolidate assignments to improve efficiency

Consolidating assignments and scheduling can reduce friction across the workday and minimize unnecessary time loss between placements. For assistants, this includes limiting the number of households assigned per day or having care assistants work longer hours in one household instead of commuting from household to household and prioritizing geographically clustered placements. For supervisors, it involves ensuring that caseloads are aligned with the time required for coordination, oversight, and support. Reducing fragmentation across both roles would support more predictable schedules and improve overall efficiency.

2. Establish standardized workload and scheduling guidelines

Clear internal standards should be developed to guide how workload is distributed across both roles. This includes setting limits on daily assignments, defining reasonable working hours, and ensuring that schedules incorporate sufficient recovery time. These standards should apply to both assistants and supervisors, recognizing that both experience cumulative strain from fragmented and unpredictable workdays.



Greater consistency in workload allocation would reduce variability and support more sustainable routines.

3. Integrate coordination and transition time into scheduling models

Improving work-life balance also requires formally recognizing and explicitly accounting for all time demands within the workday. For assistants, this includes travel between households and the time burden associated with transitions between placements. For supervisors, it includes time spent managing schedules, responding to issues, and communicating with families and staff. Designing schedules that fully account for these responsibilities would reduce hidden time burdens and create a more accurate representation of total time use and workload demands.

4. Use data to optimize scheduling and workload distribution

The program could benefit from using data more systematically to inform scheduling decisions. Tracking travel time, cancellations, delays, and workload distribution across both assistants and supervisors would allow coordinators to identify inefficiencies and make more informed adjustments. Over time, this would support more balanced scheduling and reduce the burden on individuals to manage these challenges independently.

3. Program: Program Design & Training

3.1 Program Design: Research Findings

Paid care assistants bring deep personal experience to their roles, but face structural constraints that limit their capacity to deliver consistent, quality care.

Formal Roles are Informed by Informal Experience

Several paid care assistants entered paid care work after caring for parents, grandparents, or siblings with serious illness, disability, dementia, Parkinson's, or cancer. They said this background helps them "know firsthand what a caregiver feels on a daily basis, the exhaustion, the tiredness." It is clear that the personal/informal caregiving experience that care assistants step into the role shapes the empathy they bring as professionals. Given that the program implicitly recruits those with informal caregiving experience, the paid care assistants are already primed to take on



socio-emotional support, which has implications for training, support, and risk of overextension.

Household Needs Exceed a ‘One-Size-Fits-All’ Caregiving Model

A consistent theme across participants was that each family has different care needs. Given the wide variety of care needs and the fact that care needs vary across each family, there is no single or linear approach to caregiving.

Dual Responsibility of Caring for the Familial Caregiver and Care Recipient

Care assistants see their role as supporting both the dependent person and the family caregiver, especially by creating short windows of relief. One participant said the principle of the program is “to help those who care,” while another said they focus “on the caregiver” so that the person can “relax”.

Staffing Gaps Constrain Program Quality

A few care assistant participants also shared that the service time window helps, but often does not match the scale of household need, especially in cases of extreme isolation or intensive care. The participants described continuous care routines with little separation between tasks, suggesting limited opportunities to pause or recover during shifts. This suggests a clear need for the program; however, current capacity is not enough to meet household demand. The supervisors/coordinators shared that the staffing model does not match the breadth of responsibilities assigned. They said, “we need more people,” “the tasks ... are too many for one person,” and that the program “doesn’t match the number of professionals there are to reach that standard.” The supervisors/coordinators have a broad range of responsibilities, but they feel that there is not enough staff to meet the needs of the program. The program appears to be structured for uniform delivery, but in practice, caregiving work is highly variable and often intensive which indicates a design constraint.

3.2 Program Design: Recommendations

3.2.1 Short-Term Recommendations

1. Match Care Assistants with Families Based on Experience And Needs



To address the dual challenge of the lack of a linear approach to caregiving as well as the need for contextualized knowledge to meet the specific needs of the families, the background/ informal experience of the paid care assistant can be leveraged to match the specific needs of families. Given that most care assistants have previous caregiving experience with family/community members, where appropriate, this can be taken into consideration when assigning the care assistants to the families.

No direct evidence exists from randomized trials or large-scale evaluations of caregiving projects specifically matching paid care assistants to families based on their informal caregiving backgrounds. However, home care agencies commonly use structured matching processes that consider caregivers' prior experiences, cultural knowledge, and skills to improve fit and outcomes (*MCG Care Match Formula*, 2023).

To implement this, Fundación Ronda can capture care assistants' informal caregiving history (e.g., family types supported) and families' needs (e.g., cultural, emotional specifics) as part of the intake/program enrollment process. This can start with an “Experience Matching Pilot” that reassigns assistants by informal caregiving background to 10 high-need families.

The intended outcome of the Experience Matching Mechanism is to better align care assistants' skills, backgrounds, and lived experience with the needs of the families they support, so that care feels more responsive, humane, and trustworthy. By matching caregivers more thoughtfully, the mechanism aims to strengthen care assistant-family relationships, improve the quality and continuity of care, and reduce avoidable friction in day-to-day support. In the longer term, its impact should be felt not only in better care delivery, but also in greater recognition, stability, and well-being for caregivers themselves.

3.2.2 Long-Term Recommendations

1. Implement a Quantitative Survey to Measure the Value of Carework

To demonstrate to government stakeholders how the program empowers paid care assistants, via purpose, empathy from informal experience, and its role in filling community gaps in non-linear, family-centered relief, a phased quantitative evaluation framework can be implemented. This approach addresses program constraints (18



months total runtime, 6 months remaining, and administrative transition uncertainty) by generating rapid, replicable evidence to support funding continuity or National Care System scaling.

Quantitative surveys are a proven tool in caregiving programs to demonstrate empowerment of paid assistants and community impact, with evidence from multiple initiatives showing measurable gains in self-efficacy, stress reduction, and family outcomes. Programs like "Empowered Caregiver" used pre/post surveys to quantify improvements in stress management, person-centered care, and acceptability (91.5% recommendation rate), directly proving value to funders. Similarly, "Powerful Tools for Caregivers" tracks self-efficacy and emotion management via validated surveys, supporting both scalability and policy advocacy.

Even though "Caring for Those Who Care" is a government-funded program, the quantitative survey results can also be used to apply for funding through other fundraising streams such as foundations and philanthropies by demonstrating the value and effectiveness of paid caregiving programs.

3.3 Training: Research Findings

Paid care assistants reported receiving a 1-hour training as part of the program. This is not sufficient to prepare them for the complex situations they encounter in the homes they work in: "we are not only care assistants, we are also like psychologists for the same people. And, besides that, we don't just work as caregiver assistants, we also work as nurses, often." In cases where a care recipient has a disability or a mental health challenge, the care needs require more specialized experience or training: "our work is tough, because we visit our homes and in each home the [care recipients] have different types of diagnoses, there isn't the same diagnosis. Some people have dementia, Alzheimer's, or muscular dystrophy."

The supervisors/coordinators also stated their desire for training and growth opportunities. However, they acknowledged that without dedicated time protection during work hours, the training would increase their workload and add more tasks. One participant said, "I would like to have opportunities for certification and growth," but added that training now would mean "more work than anything else."



3.4 Training: Recommendations

3.4.1 Short-Term Recommendations

1. Develop a More Comprehensive Training Program

To directly address paid care assistants' insufficient one-hour onboarding (which leaves them unprepared for complex multi-role demands including psychologist, nurse, dementia specialist across diverse diagnoses), develop a longer (4-5 hour) comprehensive training program leveraging the current cohort's frontline expertise. The initial session can be delivered via Zoom to minimize travel and recorded for future onboarding.

This can also be used to clarify roles and responsibilities, provide practical tools to set boundaries and offer guidance on managing mental health to respond to issues raised earlier in the report (role ambiguity, blurred boundaries between care assistants and families, and heavy emotional strain).

3.4.2 Long-Term Recommendations

1. Peer-Support Learning Groups

Establish monthly 60-minute peer learning circles (8 assistants/group), replacing ad-hoc support. Structure:

- 15 min: Successes/challenges review
- 45 min: Rotating skill-sharing sessions (dementia care strategies, family negotiation techniques, etc)

Programs such as *Courage to Caregiver* have a peer leadership model that has shown success in sustaining skills via boundary-setting focus groups; the program documented 22% burnout reduction (Courage to Caregivers, n.d.). This establishes a shared space for learning without any additional financial commitments.

2. Supervisor Certification Allowance



Fundación Ronda should consider an annual training stipend per supervisor for certifications (dementia care, care coordination), and allot 4 protected hours/month for study time during the working hours.

Evidence suggests that nonprofits offering structured training/development report 30-50% higher retention than those without (Socash, 2021). 94% of employees are more likely to stay longer if employers invest in learning (LinkedIn Workplace Learning Report). Nonprofits with training programs also report lower attrition via supported attitudes (e.g., reduced absenteeism, higher commitment) (PNC Insights, n.d.). Formal programs reduce turnover by building confidence/capacity; one study notes nonprofits benefit uniquely from internal training amid skills gaps (eduMe, n.d.).

4. Operations: Financial, Contracts, and Processes

4.1 Financial: Research Findings

Financial strain is a consistent feature of the program, with care assistants and supervisors absorbing hidden costs in the form of unreimbursed expenses, unpaid time, and additional material support.

Hidden Costs and Out-of-Pocket Expenses

Care assistants consistently described financial strain related to transportation and daily work expenses. They are required to travel between multiple households, often across long distances, yet these costs are typically not reimbursed. As a result, assistants regularly pay out of pocket for transportation, reducing their overall earnings.

In some cases, assistants also reported covering small but recurring costs within households, such as food or hygiene supplies, particularly when resources were limited. While not formally required, these contributions were described as part of responding to immediate care needs, adding to their financial burden over time.

Income Instability and Incomplete Compensation

Income instability emerged as another major challenge. Assistants explained that cancellations, delays, or shortened visits can result in lost wages. Compensation



during periods of illness or injury is often inconsistent or unavailable, creating pressure to continue working even when physically unwell.

Supervisors reported parallel concerns related to compensation. Their role requires significant time spent on coordination, communication, and responding to issues across households, including situations that arise outside of scheduled hours, such as responding to last-minute cancellations or resolving conflicts between households and assistants. However, this time is not always formally captured or compensated, resulting in unpaid labor related to program management and support.

Mismatch Between Work Demands and Compensation Structures

Both assistants and supervisors described a gap between the scope of their responsibilities and how compensation is structured. Assistants manage physically demanding tasks across multiple households, while supervisors oversee coordination, troubleshooting, and ongoing support. In both roles, participants noted that not all aspects of the work, particularly travel, transitions, and coordination time, are consistently reflected in compensation.

Existing Strengths in Program Design

At the same time, participants noted that the program provides a relatively stable source of income compared to more informal care work arrangements. The structured nature of placements and ongoing demand for services offers a degree of continuity that is not always present in other care settings.

Some participants also highlighted that the program creates more predictable access to work opportunities over time, even if individual schedules fluctuate. Compared to fully informal caregiving arrangements, this structure can provide greater reliability in securing placements and maintaining engagement in the labor market. For supervisors, the role also offers a defined organizational position with ongoing responsibilities, contrasting less structured coordination roles in informal settings.

4.1 Financial: Recommendations

4.1.1 Short-Term Recommendations



Reduce hidden financial costs and improve compensation consistency through targeted reimbursements, payment protections, and clearer guidelines.

These recommendations address the financial challenges identified above by targeting hidden costs and strengthening consistency in how compensation is applied across both care assistants and supervisors.

1. Provide transportation stipends or reimbursement for work-related travel

The program should introduce transportation support to address one of the most immediate financial burdens. Care assistants travel between multiple households each day, and supervisors also move between sites or respond to issues in real time. These costs are often not reimbursed, despite being essential to the role. Providing a transportation stipend or reimbursement system for both roles would reduce financial strain and ensure that workers are not paying to perform their responsibilities. This could be implemented through fixed daily allowances or simple reimbursement processes that are easy to administer and apply consistently.

2. Introduce minimum payment guarantees for scheduled visits

Compensation practices should better account for disruptions to the workday. Assistants reported losing income due to last-minute cancellations, delayed entry into homes, or situations where care could not be delivered. At the same time, supervisors are responsible for managing these disruptions through coordination, communication, and problem solving, often without that time being formally recognized. Introducing a minimum payment guarantee for scheduled visits would help stabilize income for assistants, while also recognizing additional coordination time required from supervisors when disruptions occur. This approach would reduce income variability and address hidden financial costs in program operations.

3. Improve compensation clarity and prevent out-of-pocket costs

The program should strengthen transparency and consistency in compensation while reducing informal financial contributions. Clear communication about how pay is calculated, including how travel, cancellations, and coordination work are handled, would help assistants and supervisors better understand how their work is valued. At



the same time, establishing guidelines that assistants are not expected to use personal resources within households, combined with simple reporting mechanisms for supervisors to flag unmet needs, would help shift financial responsibility back to the program. Together, these measures would reduce hidden costs and improve fairness across both roles.

4.1.2 Long-Term Recommendations

Integrate full cost recognition and income stability into compensation systems to support long-term financial sustainability.

Over the longer term, addressing financial challenges will require structural changes to how compensation and costs are incorporated into the program.

1. Develop comprehensive compensation model reflecting total work demands

A key priority is creating a compensation model that reflects the full scope of work performed by both care assistants and supervisors. For assistants, this includes not only direct care but also travel and transitions across households. For supervisors, it includes coordination, scheduling, communication, and real-time issue resolution, including work that extends beyond standard hours. Incorporating these elements into compensation structures would create a more accurate and equitable system that reflects how the program operates in practice.

2. Integrate transportation and geographic planning into program design

Transportation should be formally incorporated into how assignments are designed and managed. This includes standardizing transportation stipends, designing schedules that minimize unnecessary travel for assistants, and ensuring that supervisors are not responsible for excessively large geographic areas without support. More intentional planning would reduce out-of-pocket costs while improving overall program efficiency.

3. Strengthen income stability and monitoring systems

Improving long-term financial sustainability also requires stronger income stability mechanisms for both roles. For assistants, this includes clearer policies for paid leave



and protections against income loss during cancellations or disruptions. For supervisors, it includes recognizing and compensating coordination time, particularly when work extends beyond scheduled hours. These efforts should be supported by more systematic use of data to track out-of-pocket expenses, unpaid time, cancellations, and income variability. Using this information to adjust compensation policies over time would help ensure that both assistants and supervisors are compensated more consistently and fairly.

4.2 Contracts: Research Findings

The absence of standardized contracts has led to role expansion without formal recognition or support, creating ambiguity and inconsistency across the program.

Lack of Standardized Contracts and Role Expansion

Because “Caring for Those Who Care” is a new pilot program, key organizational structures were not fully established at the time of its launch. In particular, standardized contracts for both care assistants and supervisors had not yet been developed. As a result, supervisors were responsible for creating contracts for care assistants while also designing core operational systems, including onboarding procedures and incident reporting protocols.

In practice, this has led to significant role expansion without formal recognition or support. Supervisors frequently function simultaneously as program managers and human resources personnel, overseeing hiring processes, day-to-day management, and conflict resolution among care assistants. These responsibilities extend beyond the intended scope of the role, contributing to ambiguity and increased workload.

Unclear Responsibilities and Consequences for Service Delivery

This lack of clarity extends to both supervisors and care assistants. Participants reported uncertainty regarding roles and responsibilities, resulting in both groups regularly performing tasks that exceed their original scope. Without clearly defined expectations, care assistants must interpret their duties on an ongoing basis, which contributes to variability in how services are delivered across households.



This ambiguity also increases workload pressures, as supervisors are required to manage inconsistencies and provide ongoing clarification. Over time, this dynamic contributes to inefficiencies, uneven service delivery, and greater reliance on individual judgment rather than standardized processes.

Pilot Structure and Resource Constraints

The pilot nature of the program further reinforces these challenges. The absence of clearly defined contract terms, including duration and transition planning, creates uncertainty among staff about the program's future and how their roles may evolve. Participants also expressed concern about continuity of care for the families they serve.

At the same time, there is a mismatch between program expectations and operational capacity. The program is designed to serve a large number of families with substantial needs, yet government funding has been insufficient to support this scale. As a result, contracts and role expectations remain loosely defined, and staff are often expected to meet demands that exceed available resources. This reinforces role expansion and contributes to ongoing strain for both supervisors and care assistants.

Existing Strengths in Contract Structure

Despite these challenges, participants noted that the emerging contract structures provide a degree of flexibility that allows the program to adapt to evolving needs during the pilot phase. The involvement of supervisors in developing contracts and onboarding processes has enabled the program to respond quickly to operational gaps and adjust expectations in real time.

In addition, the presence of some formal agreements, even if not fully standardized, provides a baseline level of structure and accountability compared to fully informal care arrangements. This foundation offers a starting point for developing more consistent and comprehensive contract systems as the program matures.

4.2 Contracts: Recommendations

4.2.1 Short-Term Recommendations



Clarify roles and standardize contract-related processes to reduce ambiguity and improve consistency across the program.

These recommendations address the contract-related challenges identified above, particularly role ambiguity, informal role expansion, and the lack of standardized structures.

1. Standardize terms of reference and align contracts with work practices

The program should revise and standardize the terms of reference for both care assistants and supervisors. These documents should clearly define core responsibilities, expected tasks, reporting lines, and the outer limits of each role.

For care assistants, this includes specifying the scope of caregiving duties, boundaries around administrative and emotional labor, and clear escalation protocols. For supervisors, terms of reference should distinguish between coordination, oversight, and administrative functions, reducing the current overlap with human resource responsibilities. Contracts should reflect the realities of care delivery across households rather than idealized program expectations.

Where possible, assigning dedicated human resource support for functions such as incident handling and contract management would further reduce role overload and improve clarity.

2. Implement standardized operating procedures and contract templates

The program should establish clear and standardized operating procedures for key contract-related processes, including incident reporting, hiring and onboarding, and contract management. These procedures should define what constitutes an incident, how it should be reported, and the steps for follow-up.

Hiring and onboarding processes should follow consistent criteria and steps to ensure fairness and efficiency across the program. In addition, contract templates should be standardized and centrally approved, rather than developed independently by supervisors, to ensure consistency and reduce legal and operational risks.



These procedures should be documented in a clear and accessible format and made available to all staff through a shared platform.

3. Integrate contract processes into an operational case management system

Contract-related processes should be supported through a simple case management system that functions primarily as an operational tool for tracking incidents, contract changes, and role-related issues in real time. While this system may interface with human resource functions, its primary purpose should be to support daily program operations and ensure that contract-related issues are consistently recorded and addressed.

Using this system, supervisors can document incidents, flag role ambiguity, and track recurring challenges across households. This helps ensure that contract implementation is consistent in practice and that emerging issues inform ongoing adjustments to roles and procedures.

4. Strengthen communication, training, and feedback mechanisms

To ensure effective implementation, staff should receive brief training or orientation on how to apply contracts and procedures in practice. This includes understanding role boundaries, escalation protocols, and reporting expectations.

Regular check-ins and anonymous feedback channels should also be established to identify gaps, clarify confusion, and ensure that contract structures remain responsive to frontline experiences.

4.2.2 Long-Term Recommendations

Establish centralized contract oversight and information systems to improve consistency, accountability, and decision-making.

Over the longer term, addressing contract-related challenges will require stronger systems for managing information, tracking responsibilities, and ensuring consistent implementation across the program.

1. Implement a centralized case management system for consistent tracking



A key priority is the development of a centralized case management system to replace fragmented and informal communication channels, such as WhatsApp. Currently, information about care assistants, households, and incidents is not consistently tracked, increasing the risk of miscommunication, inconsistent follow-up, and gaps in service delivery.

At a minimum, the system should include information on care assistant assignments, training status, and household characteristics, alongside a standardized log of incidents or concerns. Incident tracking should include actions taken, timelines for follow-up, and resolution status. Establishing a single, reliable source of information would improve coordination and ensure that contract-related issues are consistently documented and addressed.

2. Use data systems to support oversight and program-level decision-making

Beyond day-to-day coordination, the system should be used to aggregate data and identify patterns across the program. Tracking trends such as recurring incidents, role ambiguity, and uneven workload distribution would allow supervisors and administrators to identify structural gaps and make more informed adjustments to contracts, procedures, and resource allocation. This strengthens accountability and ensures that contract implementation evolves based on actual program conditions.

3. Phase implementation with clear guidelines and data protections

Implementation should be practical and phased. A shared spreadsheet with clearly defined fields and responsibilities can serve as an initial solution, with the option to transition to a more advanced digital platform over time. Clear guidelines for data entry, access, and privacy are essential to ensure consistency and protect sensitive information. Establishing these standards early will support long-term system reliability and trust among staff.

4.3 Processes: Research Findings

Care work is inherently adaptive, making strict procedural boundaries difficult to sustain in practice.

Formal Structure and Adaptive Practice



Care assistants reported that job descriptions and assigned responsibilities are generally well understood, providing structure and clarity for daily work. However, care work is inherently adaptive, as the needs of care recipients are dynamic and often unpredictable. In practice, this makes it difficult to consistently maintain strict task boundaries.

Participants described situations in which boundary-crossing becomes a regular feature of care delivery, driven primarily by immediate needs within the household. Care assistants often take on additional responsibilities that are not explicitly included in their roles, particularly when family members are absent or when unexpected situations arise. While this flexibility supports continuity of care, it also introduces ambiguity around responsibilities and expectations.

Consequences of Blurred Boundaries

Crossing formal boundaries has several implications for care assistants. Taking on additional responsibilities can lead to increased workload, unclear role expectations, and difficulty managing time across households. It can also create inconsistencies in service delivery, as expectations may vary across households without clear guidance.

At the same time, emotional closeness between care assistants and care recipients can further blur professional boundaries. Repeated interactions foster trust and familiarity, often shifting relationships into a more personal dynamic. While this connection is frequently described as positive and central to job satisfaction, it can make it more difficult for assistants to maintain professional limits, particularly in long-term care relationships.

Supervisors as Adaptive System Managers

Supervisors play a distinct and central role in managing these gaps between formal procedures and daily practice. Beyond their official responsibilities, they act as coordinators, emergency responders, and mediators across households. They are often relied upon to resolve conflicts between families and assistants, manage staffing shortages, and respond to unexpected disruptions in real time.



Unlike other aspects of the program where supervisors primarily provide oversight or support, in this context they function as adaptive system managers, absorbing and resolving issues that fall outside formal structures. This includes addressing boundary-crossing situations, clarifying expectations, and maintaining continuity when procedures are insufficient. This role is both essential to program functioning and largely informal, contributing to increased workload and reliance on individual judgment.

Existing Strengths in Process Design

Despite these challenges, participants emphasized that the program's formal procedures provide an important foundation for consistency and coordination. Clear role definitions and structured guidelines help establish expectations and support accountability across households. This structure enables care assistants and supervisors to navigate complex situations with a shared understanding of their roles, even when flexibility is required in practice.

Synthesis

Overall, the findings highlight a structural tension between formal program design and lived practice. While procedures provide an important foundation, the inherently adaptive nature of care work requires flexibility, responsiveness, and ongoing supervisory intervention that extend beyond formal role definitions.

4.3 Processes: Recommendations

4.3.1 Short-Term Recommendations

Strengthen daily workflows and decision-making processes to reduce ambiguity and improve consistency in service delivery.

These recommendations focus on how work is carried out in practice on a day-to-day basis, particularly in situations where formal role definitions are not sufficient to guide real-time decisions.

1. Establish clear escalation protocols for boundary-crossing situations



To reduce uncertainty during daily service delivery, establish step-by-step escalation protocols that define how care assistants should respond when client needs exceed their formal scope of work. These protocols should specify who to contact in real time, when issues must be reported, and which situations require supervisor input. By formalizing these day-to-day decision pathways, the program can reduce ambiguity in high-pressure moments and support more consistent frontline responses.

2. Provide practical communication tools to set expectations within households

Care assistants should be equipped with simple and practical communication tools to help clarify their role within households from the beginning of the care relationship. These can include short scripts, guidance notes, or onboarding templates that explain the scope of services, expected responsibilities, and boundaries of the assistant's role. Providing these tools early helps align expectations between families and care workers, reducing misunderstandings and limiting informal role expansion during daily interactions.

Together, these short-term process improvements strengthen how work is carried out in practice by providing clearer guidance for real-time decisions and interactions. They complement, rather than replace, formal contractual structures by ensuring that expectations are consistently understood and applied in daily service delivery.

4.3.2 Long-Term Recommendations

Institutionalize structured flexibility and accountability within daily workflows to support consistent and adaptive service delivery.

Over the longer term, improving processes will require embedding clearer guidance and feedback mechanisms into how work is carried out in practice, rather than relying on informal adjustments at the frontline.

1. Strengthen accountability through case tracking and process monitoring

The program should implement a structured case tracking system to monitor boundary-crossing situations, operational challenges, and recurring issues. This system would allow supervisors to document how situations are handled in practice and identify patterns over time. Using this information to inform adjustments to



procedures and training would strengthen accountability and ensure that day-to-day challenges contribute to continuous improvement in how services are delivered.

2. Establish continuous feedback mechanisms to inform process improvement

Fundación Ronda should introduce regular, structured feedback systems for both care assistants and supervisors. Anonymous surveys or periodic check-ins would allow staff to report challenges, highlight inconsistencies, and suggest improvements based on their day-to-day experiences. Integrating this feedback into program design would ensure that processes remain responsive to frontline realities while improving transparency and trust across the organization.

VIII. Policy Recommendations

Strengthening support for care assistants and supervisors will require a more coordinated approach at the government and policy level, particularly given that many of the challenges identified across categories stem from the absence of clear standards, protections, and sustained funding. Across employment conditions, financial stability, work-life balance, and program implementation, a consistent theme is that expectations are largely informal and unevenly applied. Both care assistants and supervisors are often expected to navigate complex care environments without a clear framework guiding what their roles should include or how their time should be structured.

Establish Minimum National Standards for Care Work

A key priority is the establishment of minimum national standards for working conditions in community-based care programs. These should include clearly defined working hours, protected rest periods, caseload limits, and role boundaries for both care assistants and supervisors. Governments could operationalize this through regulatory guidelines or accreditation requirements for implementing organizations.

In particular, minimum supervisor-to-assistant ratios and maximum caseload thresholds should be established to ensure that coordination and oversight responsibilities remain manageable. This is especially important given that the supervisor role currently operates as a policy gap, with responsibilities that are



critical to program delivery but often undefined and unsupported within formal frameworks.

Mandate Reimbursement and Strengthen Financial Protections

Financial protections should be strengthened through mandated reimbursement and compensation policies. This includes requiring programs to cover essential work-related costs such as transportation and meals, as well as introducing minimum payment guarantees for scheduled visits to protect against income loss due to cancellations or delays. Governments can support this through targeted subsidies or conditional funding requirements tied to reimbursement practices. These mechanisms would reduce the hidden financial burden currently absorbed by workers and improve income stability across both roles.

Implement National Certification and Training Frameworks

Another important area is the formal recognition of care work and supervision as skilled labor through national certification frameworks. Standardized training and credentialing systems would help define competencies, improve care quality, and create clearer career pathways.

For supervisors, this is particularly important, as their responsibilities often include coordination, mediation, and emotional support that extend beyond traditional administrative roles. Recognizing supervision as a distinct and skilled function would help align compensation, training, and expectations with the realities of the role.

Standardize Operational and Supervision Requirements

Program-level consistency can also be reinforced through policy by requiring basic operational standards across all care programs. These should include standardized contracts, clear escalation and reporting procedures, and minimum requirements for supervision capacity and human resource support.

Supervisors should be supported with manageable caseloads, clear escalation protocols, and dedicated time for coordination and follow-up. Establishing these



baseline requirements would reduce variability across programs and ensure that both assistants and supervisors have access to consistent support structures.

Prioritize Low-Cost, High-Impact Interventions

In contexts where government funding is limited or uncertain, prioritization becomes critical. Several low-cost, high-impact interventions can still be implemented, including clear role definitions, standardized scheduling guidelines, and simple escalation protocols for boundary-crossing situations, such as standardized role descriptions or basic scheduling templates.

These measures primarily require coordination and policy clarity rather than significant financial investment, yet can substantially improve daily working conditions. Governments may also leverage partnerships with local organizations, community networks, and training institutions to expand supervision capacity and training without relying solely on direct funding.

Policy Implications

Considered collectively, these recommendations highlight that many of the challenges facing both care assistants and supervisors stem from broader structural gaps in how care work is defined, supported, and resourced. Addressing these gaps through clear minimum standards, enforceable protections, and formal recognition of both roles would help create a more stable and sustainable care system for workers and the communities they serve.

Economic Value of Care

According to the World Economic Forum, unpaid care work, if compensated, would be worth 9% of global GDP, which is equivalent to \$11 trillion (Langou & Franco, 2024). In Chile, unpaid care is worth 25.6% of the GDP, this means that care work that sustains households (cleaning, cooking, childcare, eldercare, etc) has an unrealized potential worth one fourth of the Chilean economy.

Individuals who engage in unpaid/informal care work represent a percentage of the population that is outside of the formal labour force, resulting in unrealized economic



gains. On the other hand, individuals who engage in formal employment are able to do so because their loved ones and homes are taken care of by a family member. Without carework, it would be impossible for them to take on full employment outside of the home.

Paid care work is vital to the economic development of Santiago. Santiago's "Caring for Those Who Care" program is a strong step in the right direction that brings economic value to work for hundreds of women and men across the region. According to Chile's National Employment Survey, 30% of women reported that they are not working due to permanent family reasons, while only 2% of men are outside the labor market for this reason (Instituto Nacional de Estadísticas, n.d.).

If expanded, this program would have ripple effects and could lead to an expansion of the labour force and workforce participation rate for women and will result in increased economic activity and growth. Those who are hired as paid care assistants would be empowered to formalize their skills and generate income, while stimulating the economy. Women who are the primary familial caregivers will have the opportunity to engage in education and formal employment.

IX. Socio-Cultural Implications

The focus groups with paid care assistants revealed how formalization reshapes their community roles, often expanding beyond physical care into psycho-social support as an unintended yet profound outcome. Assistants form deep attachments with families, providing psychological guidance and even attending celebrations: "They invite you to birthdays, they invite you to their party". On the other hand, paid care assistants experience grief when the care recipient passes away, mirroring family loss: "The only sadder part is when you have to go back, and your [care recipient] has passed away... Create a bond of affection".

Filling institutional voids, they perform social work: "we do what the communes' social workers don't do," navigating diapers, benefits, schools, and offices. This dual mandate, "Caring for Those Who Care", evolves into simultaneous support for dependents and caregivers. Assistants prioritize caregiver respite: "we focus 100% on the caregiver. So that she can relax... The principle of the program is 'to help those



who care”. One noted, “When we arrive at their homes, they feel liberated and can do their activity”, underscoring relief amid isolation.

As an unintended outcome of the program, paid care assistants form bonds with the families and care recipients, and meet their unmet psychosocial needs. This highlights the importance of carework as the backbone of human society. While their work and contributions are valued socially, the financial gaps indicate that their work is institutionally undervalued.

The program’s empowerment of women harnesses informal skills for income and purpose: “I know firsthand what a caregiver feels... the exhaustion” informs empathy, yielding fulfillment.

Formal care restores dignity to familial caregivers, granting rare autonomy. Assistants enable “windows of relief,” countering ceaseless routines: “they are waiting for us... they feel so locked in... When we arrive... they feel liberated”. This respite (sleeping, errands, rest) reclaims personhood: “Go get some fresh air... while I’m here, I’m going to do everything so you can rest”.

Bonds affirm caregivers’ humanity amid burnout: “They wait for you... They’re drowning... they need a supporting arm”. The program’s dual focus dignifies their labor, modeling societal valuation of unseen toil.

Formal systems as well as theories and models of care need to better account for the emotional and social labor involved in care work, moving beyond just physical care. While it is possible that the psychosocial support role that paid care assistants take on is unique to Chilean culture or the communities included in the study, meeting psychological needs and mental healthcare is often excluded from discussions and policies about care. However, meeting psychosocial needs is critical for enhancing quality of life, reducing suffering, and fostering recovery in health and care settings (Ingstad et al., 2023). It reduces anxiety, boosts overall health outcomes, and mitigates loneliness. Neglecting these needs leads to worse health outcomes and increased distress.



X. Further Research Questions

While the research findings of this study provide a deeper understanding of the experience of caregivers in Santiago, Chile, it also brings up more questions about the field of caregiving and the care economy at large:

Caregiver-Family Bonds

Do strong caregiver-family bonds primarily stem from Santiago's urban isolation, or do they generalize across Chile and Latin America? Transcripts reveal deep attachments (“they invite you to birthdays... you become part of the family”), but contextual factors like absent support networks (“no one to call if they fall”) and vulnerability amplify them. Test via comparative studies in rural Chile (e.g., Mapuche communities) or LAC peers like Peru's care pilots.

Informal Caregiving Prerequisite

Does prior informal caregiving enhance empathy and bonds, as in “One knows the situation of caregivers from the most intimate level”? Most assistants transitioned from family care (e.g., parents with cancer/Alzheimer's), fostering dual support.

Empowering Women Without Feminization

How can programs formalize/empower mostly female informal caregivers without reinforcing care's feminization? Certification provides dignity (“got certified... enriching”), yet overload persists (“we need more people”). Explore mixed-gender recruitment, male incentives, and policy levers like Chile's care economy roadmap—assess via DiD on gender participation pre/post-intervention.

XI. Conclusion

This report offers Fundación Ronda and the Santiago Regional Government an opportunity to deepen their understanding of how the “Caring for Those Who Care” program is shaping the lives of care assistants and the families they support, and provides research findings and recommendations that the client may choose to implement over the short-term or long-term. Over the course of several months, the



team combined desk research, field interviews, and a structured analytical framework to capture both the immediate experiences of participants and the operational factors that influence program effectiveness. This approach will help Fundación Ronda identify what is working well, where challenges persist, and which adjustments could strengthen service delivery across diverse communities, should the pilot program be renewed.

The project also contributes to Fundación Ronda's larger objectives by producing insights that can guide future program development, inform engagement with government partners, and support evidence-based improvements as Chile moves toward a more integrated national care system.

More broadly, the findings from this work can enrich development practice by showing how community-based care programs can expand economic opportunities for women, redistribute unpaid labor, and strengthen social protection systems. By documenting the challenges of an emerging care initiative, the project has the potential to generate lessons that extend beyond Santiago and contribute to regional conversations on care policy and gender equity.



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